

EMERGENCY FIRST AID: HOW TO SAVE A CHOKING VICTIM

RECOGNITION & IMMEDIATE ACTION



TOTAL AIRWAY OBSTRUCTION:

Air movement is completely blocked. Victim struggles to breathe, cannot speak or cough.



THE 3-STEP PROTOCOL:

1. **ALERT** (Call Emergency Services/14)
2. **PROTECT** (Ensure Safety)
3. **RESCUE** (Begin Maneuvers Immediately)



The Universal Choking Sign.

Unlike partial obstruction, the victim makes **NO** audible sound while gasping.

TECHNIQUES FOR ADULTS AND CHILDREN (>1 YEAR)

STEP 1: 5 BACK SLAPS



Stand to the side, lean victim forward, head lower than shoulders. Deliver up to 5 firm blows with heel of hand.

STEP 2: THE HEIMLICH MANEUVER



If back slaps fail, stand behind, wrap arms, place fist above navel (below ribs). Perform 5 quick inward and upward thrusts.



EXAMPLE: VICTIM LYING DOWN. Kneel over, place heel of hand on upper abdomen, perform quick upward thrusts.

SPECIAL CASE: INFANTS (<1 YEAR) – MOFENSON METHOD

STEP 1: BACK SLAPS FOR INFANTS



Lay face down on forearm, resting on thigh, head lower than chest. Support jaw with fingers, avoid throat pressure.

STEP 2: 5 CHEST COMPRESSIONS



If object remains, turn infant onto back. Use two fingers to press 5 times on lower half of breastbone (between nipples).

KEY FINDING: AVOID ABDOMINAL THRUSTS.

Never use on infants due to fragile liver/spleen; use chest compressions instead.

EVALUATION & CRITICAL WARNINGS



SIGNS OF SUCCESS:

Object is expelled, victim coughs, or spontaneous breathing resumes.



FAILURE TO EXPEL:

If obstruction persists, continue alternating 5 back slaps and 5 abdominal thrusts/chest compressions until object is freed or victim loses consciousness.



LOSS OF CONSCIOUSNESS:

If victim becomes unconscious, **IMMEDIATELY** transition to Cardio-Pulmonary Resuscitation (CPR).



EXAMPLE: WARNING: PARTIAL OBSTRUCTION.

If the victim can cough or speak slightly, **DO NOT** perform maneuvers. Encourage them to cough and seek medical advice; maneuvers could worsen the block.

Life-Saving Essentials: Airway Management and Artificial Respiration

A step-by-step guide to restore breathing in unconscious victims.

PHASE 1: AIRWAY LIBERATION (CLEARING THE PATH)



Identify the Obstacle

Recognize that in comatose victims, the airway is often blocked by the tongue, secretions, or blood.



Head Tilt and Hyper-extension

Place one hand on the forehead and the other under the neck to tilt the head back, opening the respiratory path.



Jaw Sub-luxation and Mouth Check

Lift the jaw and open the mouth to verify it is empty and clear of any visible obstructions.

PHASE 2: MOUTH-TO-MOUTH VENTILATION (ARTIFICIAL RESPIRATION)



Position the Victim

Ensure the patient is in "Décubitus dorsal" (lying flat on their back) with the neck maintained in hyper-extension.



Pinch the Nostrils

Use the fingers of the hand on the forehead to pinch the nose closed to prevent air from escaping.



Deliver Insufflations

Take a deep breath, seal your mouth around the victim's, and blow four complete breaths into the lungs without letting them deflate between turns.



Monitor for Success

Watch for chest and abdominal movements while listening for breath sounds with your ear near the victim's face.

PHASE 3: MOUTH-TO-NOSE ALTERNATIVE



When to Use Mouth-to-Nose

This technique is used as an alternative to mouth-to-mouth while following the same basic airway opening principles.



Seal the Lips

Use your thumb to press the victim's lips together, ensuring the mouth is completely closed.



Nasal Ventilation

Place your mouth over the victim's nose with your cheeks pressed against their lips and blow as you would for mouth-to-mouth.